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Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

PRESIDENTIAL EBOLA TASK FORCE 17

MVE17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°171/MVE-BDBV/01/11/2026

Reporting date: November 1, 2026 Publication date: November 2, 2026

HIGHLIGHTS (last 24 hours)

For November 1, 2026, sixty-five (65) new confirmed cases and 23 deaths (20 community deaths and 3 in Ebola Treatment Centers) of Ebola Virus Disease (EVD) were reported in the provinces of North Kivu (39), Ituri (19), and Haut-Uélé (7). A total of 464 new samples were analyzed, 65 of which tested positive for the Bundibugyo virus, representing a positivity rate of 14.0%. No new health zones were affected in the last 24 hours.

ACCUMULATIVE CASE	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
10532	5126	48.7%	961	2,834	84.0%

Affected provinces and affected health zones

8 Provinces affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo, Bas-Uélé, South Ubangi, and North Ubangi

67 out of 178 health zones affected (37.6%)

Summary of key points from the last 24 hours

At the end of the day on November 1, 2026, sixty-five (65) new confirmed cases of Ebola virus disease (EVD) were reported in three provinces: North Kivu (39 cases), Ituri (19 cases), and Haut-Uélé (7 cases). Among these cases, 20 community deaths were reported (11 in North Kivu, 8 in Ituri, and 1 in Haut-Uélé). In addition, 3 confirmed deaths were recorded in Ebola Treatment Centers (ETCs) (2 in North Kivu and 1 in Ituri), bringing the total number of confirmed deaths reported during this period to 23.

In addition, 10 patients (4 in Ituri, 4 in North Kivu and 2 in Tshopo) were declared cured after obtaining two negative results in the control test for Ebola virus disease.

Since the start of the epidemic, the DRC has recorded 10,532 confirmed cases, including 5,126 deaths, representing an overall case fatality rate of 48.7%. Ituri remains the epicenter of the epidemic, accounting for 70.6% of cumulative confirmed cases and 29.2% of newly reported cases. North Kivu represents 60.0% of new cases and has a case fatality rate of 55.4%, higher than the national average.

No new health zones have been affected in the last 24 hours.

Thus, 67 out of 178 health zones (37.6%), spread across eight provinces, have been affected since the start of the epidemic.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of November 1, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	19	7431	3,492	47.0%	28/36 (77.8%)
North Kivu	39	2518	1,395	55.4%	17/34 (50.0%)
Haut-Uélé	7	467	205	43.9%	7/13 (53.8%)
Tshopo	0	92	27	29.3%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	13	4	30.8%	4/11 (36.4%)
South Ubangi	0	2	1	50.0%	1/16 (6.3%)
North Ubangi	0	6	1	16.7%	2/11 (18.2%)
Total	**65**	**10,532**	**5,126**	**48.7%**	**67/178 (37.6%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of November 1, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**7431**	**3492**	**47.0%**	**19**	**8**	**1**	**9**
Adja	11	1	9.1%				0
Aru	25	7	28.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	220	49	22.3%				0
Boga	2	2	100.0%				0
Bunia	2012	701	34.8%	5	1	1	2
Damascus	41	10	24.4%				0
Drodro	15	7	46.7%				0
Fataki	84	31	36.9%	0			0
Gethy	11	7	63.6%				0
Kambala	2	0	0.0%				0
Kilo	40	13	32.5%				0
Komanda	193	118	61.1%				0
Lita	331	165	49.8%	2	0	0	0
Logo	14	9	64.3%				0
Lolwa	37	11	29.7%				0
Mahagi	5	1	20.0%				0
Mambasa	105	57	54.3%	4	4	0	4

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Mandima	491	264	53.8%	3	2	0	2
Mangala	398	172	43.2%				0
Mongbwalu	722	348	48.2%				0
Nia-Nia	343	198	57.7%	2	1	0	1
Nizi	855	312	36.5%	2	0	0	0
Nyankunde	139	43	30.9%				0
Rimba	25	6	24.0%				0
Rwampara	1204	433	36.0%	1	0	0	0
Tchomia	86	27	31.4%				0
To ventilate	N / A	493	N / A				0
North Kivu	**2518**	**1395**	**55.4%**	**39**	**11**	**2**	**13**
Blessed	720	476	66.1%	17	9	0	9
Good	26	9	34.6%				0
Butembo	344	201	58.4%	1	1	0	1
Goma	1	0	0.0%				0
Kalunguta	99	44	44.4%	2	0	0	0
Katwa	678	416	61.4%	1	0	1	1
Kayna	3	2	66.7%				0
Kyondo	46	22	47.8%				0
Lubero	43	15	34.9%				0
Mabalako	116	39	33.6%				0
Alimbongo	6	4	66.7%				0
Manguredjipa	16	5	31.3%				0
Masereka	43	13	30.2%	2	0	0	0
Musienene	162	77	47.5%	1	0	0	0
Mutwanga	66	15	22.7%	11	1	0	1
Oicha	93	38	40.9%	4	0	0	0
Vuhovi	56	19	33.9%	0	0	1	1
Upper Uele	**467**	**205**	**43.9%**	**7**	**1**	**0**	**1**
Boma Mangbetu	46	19	41.3%				0
Dungu	1	1	100.0%				0
Gombari	3	1	33.3%				0
Isiro	115	53	46.1%	1	0	0	0
Pawa	116	65	56.0%	3	0	0	0
Rungu	9	5	55.6%	2	1	0	1
Wamba	177	61	34.5%	1	0	0	0
Tshopo	**92**	**27**	**29.3%**	0	0	0	0
Bafwasende	18	3	16.7%				0
Kabondo	20	2	10.0%				0
Lubunga	1	0	0.0%				0
Makiso - Kisangani	37	14	37.8%				0

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Mangobo	10	7	70.0%				0
Tshopo	5	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	**3**	**1**	**33.3%**	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	**13**	**4**	**30.8%**	0	0	0	0
Buta	1	1	100.0%				0
Viadana	6	1	16.7%				0
Dakwa	1	0	0.0%				0
Ganga	5	2	40.0%				0
South Ubangi	**2**	**1**	**50.0%**	0	0	0	0
Bulu	2	1	50.0%				0
North Ubangi	**6**	**1**	**16.7%**	0	0	0	0
Wapinda	3	1	33.3%				0
Yakoma	3	0	0.0%				0
Total	**10532**	**5126**	**48.7%**	**65**	**20**	**3**	**23**

* **An exercise to harmonize non-inventory data was carried out on October 29, 2026 between the Data Management pillar and the SGI and the provincial of Ituri. This exercise led to adjustments in the data reported in the province.**

Table 3. Status of alerts notified by province for November 1, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated ed	Number of suspects No investigated ed	Number of suspects investigated and transferred At CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased			
Haut-Uélé	77	1	78	6	1	71	0	5	2	4
Bas Uélé	103	0	103	7	0	96	0	7	0	0
Ituri	1,327	54	1,381	109	54	1,218	0	160	3	93
North Kivu	1061	50	1111	114	50	947	0	164	0	99
South Ubangi	25	0	25	0	0	22	0	0	0	0
North Ubangi ND		ND	ND	ND	ND	ND	ND	ND	ND	ND
Tshopo	60	0	60	1	0	59	0	1	0	1
Total General	**2,653**	**105**	**2,758**	**237**	**105**	**2,413**	**0**	**337**	**5**	**197**

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Figure 1. Evolution of confirmed cases by date of symptom onset, from October 12 to November 1, 2026

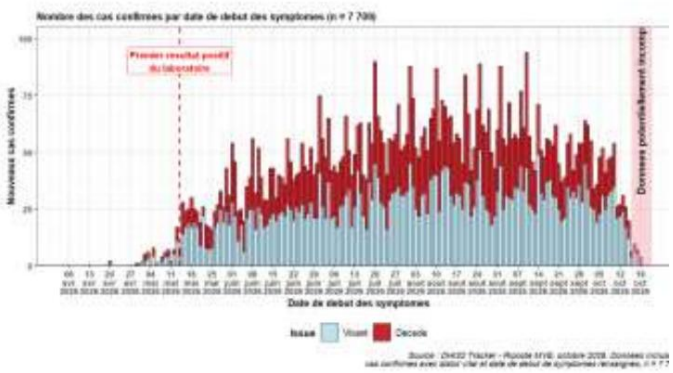
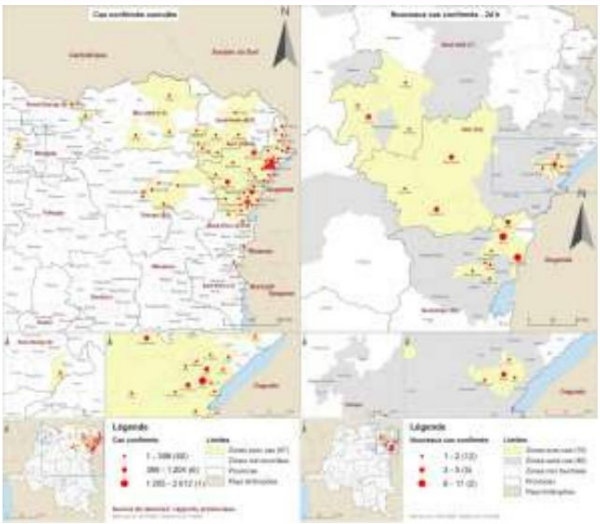


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

• Support for response activities in the affected provinces through the technical pillars; in Bas-Uélé, regular holding of SGI meetings, participation in coordination meetings with the Bunia team and press briefing by the provincial Minister of Health, Acting Governor, on the evolution of Ebola, in a context of a general strike by nursing providers throughout the province; in Tshopo, response activities remain suspended in the Bafwasende Health Zone since October 24, 2026.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.2. Main Actions

As of November 1, 2026, 2,758 alerts had been reported in the six provinces that submitted their data (Bas-Uélé, Haut-Uélé, Ituri, Nord-Kivu, Sud-Ubangi, and Tshopo), including 2,653 alerts of living persons and 105 of deaths; 2,755 were verified (99.9%). Of these, 342 (12.4%) were validated as suspected cases and 2,413 (87.6%) were invalidated. Of the 342 validated suspected cases, 337 (98.5%) were investigated and 197 were transferred to treatment centers (Table 3).

In total, 36,126 contacts were under follow-up in the six provinces that submitted their data; 30,362 were seen in the last 24 hours, representing a follow-up rate of 84.0%, below the required threshold of 95.0% and down from the previous day (86.6%). By province, the follow-up rate is 100% in Bas-Uélé (4/4), 97.3% in Sud-Ubangi (109/112, follow-up period ended on October 13), 90.8% in Ituri (13,768/15,155, of which only 78.7% were in Bunia), 87.2% in Haut-Uélé (1,288/1,477, Wamba not having reported), 79.0% in Nord-Kivu (14,604/18,494, of which 64.7% were in Beni and none of Kayna's 141 contacts were seen) and 66.6% in Tshopo (589/884), where health staff strikes and attacks against teams continue to disrupt follow-ups (Table 4).

Table 4. Status of contact tracing by province, as of November 1, 2026

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri	15,155	13,768	90.8%
Haut-Uélé	1,477	1,288	87.2%
North Kivu	18,494	14,604	79.0%
Tshopo	884	589	66.6%

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Bas-Uélé	4	4	100.0%
South Ubangi	112	109	97.3%
North Ubangi	N / A	N / A	N / A
Total	36,126	30,362	84.0%

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

For the day of November 1, 2026, five (5) alerts were notified to the PoE/PoC, all in Ituri, at Base Yedi (Komanda), Mudzi Bala (Bunia), Irumu centre (Nyankunde), Foner Mambasa (Mambasa) and Sans Plainte (Ariwara); the 5 alerts were validated and investigated within 24 hours (100%) and 4 out of 5 (80%) were transferred, 3 to the CTEs of Komanda, Bunia and Nyankunde and 1 to the ESS of Ombayi, the traveler from Foner Mambasa having refused his transfer.

Table 5. Status of key indicators for PoE/PoC by province, as of November 1, 2026

Indicators	Ituri	North-Kivu	South Kivu	Tshopo	Bas-Uélé	Overall
Completeness of the of reports	97% (60/62)	95% (20/21)	56% (5/9)	56% (10/18)	45% (10/22)	79.5% (105/132)
Strategic PoC/PoE	1	N / A	N / A	N / A	N / A	1
Number of past travelers	145,193	84,546	11,054	12,104	2,909	255,806
Proportion of travelers screened	96%	97%	99%	99%	98%	97%
% of travelers who washed their hands	97%	97%	99%	98%	99%	97%
% of travelers made aware	99%	97%	100%	100%	67%	98%
Number alerts notified	05	00	00	00	00	5
Number of validated live alerts	05	00	00	00	00	5
Number of lifeless bodies intercepted	00	00	00	00	00	0
% of suspected cases transferred to CT/CTE	80% (4/5)	0%	0%	0%	0%	80% (4/5)
% of lifeless bodies swabed/ secured	0%	0%	0%	0%	0%	0%
% of suspected cases investigated within 24 hours	100% (5/5)	0%	0%	0%	0%	100% (5/5)

2.2.2. Challenges

In North Kivu, the Butuhe PoC remains closed since the security incident of September 26, 2026, and 2,470 travelers (2.9%) refused to wash their hands; in **South Kivu,** completion remains low (56%; 5/9), and the PoEs of Kavimvira, Kalundu Port, and Baraka do not have isolation facilities; in Bas-Uélé, completion has fallen to 45% (10/22) due to lack of connectivity and mobility, and because of the strike, and only 67% of travelers were made aware of the situation; in Tshopo, completion is only 56% (10/18), with 8 private ports being closed on Sundays, and 2.0% of travelers refused to wash their hands; **in Haut-Uélé,** 5 PoCs did not report due to network outages, and the Magambe PoC is shut down due to the service provider strike.

2.3. Laboratory

2.3.1. Main actions

- **Ituri:** 19 new positive results (11 living and 8 dead) out of 168 new samples analyzed (positivity: 11.3%).

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- **North Kivu:** 39 new confirmed cases (28 living and 11 deaths) retained by surveillance; the laboratories of Beni and Butembo analyzed 241 new samples, of which 39 were positive (positivity: 16.2%).
- **Tshopo:** 9 samples from living people were analyzed (2 new suspects and 7 follow-up samples), with no new positive results (positivity: 0%).
- **Haut-Uélé:** 7 new positive results (6 living and 1 dead) out of 35 new samples analyzed at the Isiro laboratory (positivity: 20.0%); the 4 follow-up samples came back negative and 1 out of 5 Radi-One machines is out of order.

- **Bas Uélé:** 11 samples from living people were analyzed (4 at the Buta laboratory and 7 from the Poko health zone at the Isiro laboratory), all negative (positivity: 0%).

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 16 rings opened out of 37 (43.2%), 19 households decontaminated out of 27 (70.4%) and 4 FOSA out of 8 (50.0%); 49 EDS carried out out of 73 alerts, including 11 postponements (67.1%), 49 bodies swabed with 11 swab failures; the CREC facilitated the decontamination of 48 households out of 48 (100%), including 2 structures, as well as ESS and vehicles.

- **North Kivu:** 14 rings opened out of 22 (63.6%), 8 households decontaminated out of 13 (61.5%) and 10 out of 15 ESS (66.7%); training and capacity building: 285 service providers briefed, 149 ESS monitoring and support and 4 PCI-WASH score card assessments in 4 ESSs in Beni, Musienene, Lubero and Kalunguta (average score: 40.4%); 2 ESSs in Katwa equipped with PCI kits, 149 DLM functionality monitoring, 2 triage units installed in Butembo and Kyondo and 5,360 liters of water provided in public places in Katwa and Oicha; 37 schools equipped in Butembo and 1 school decontaminated in Beni; 60 EDS carried out out of 72 alerts (83.3%), 70 bodies swabed.

- **Tshopo:** decontamination of the Neema Health Center around a suspected case, of 2 households around the 85th, 87th and 92nd cases (Kabondo and Makiso-Kisangani) and of the Maleke school complex around the 83rd case; training and capacity building: 192 providers briefed on IPC themes during the follow-up of 17 out of 17 health centers in 5 health zones (100%) and initial assessment of the Pekis Health Center (score: 24%); cumulatively, 59 out of 107 health centers (55.1%) received IPC kits, 49 out of 107 (45.8%) have a triage circuit and 51 out of 54 households (94.4%) were decontaminated within 48 hours.

- **Haut-Uélé:** 1 ring open out of 1 (100%) in Wamba around a confirmed PPL, 7 households out of 7 and 2 meeting rooms decontaminated and 1 ESS out of 1 (100%) in Wamba; 6 nurses from Wamba briefed on the functionality of DLM and 3 ESS (HGR, CTE and CH Matari) equipped with PCI kits and having functional triage units; 2 EDS carried out on 2 alerts (100%) in Isiro and Pawa and 1 body swabé.

- **Bas-Uélé:** monitoring and support of 7 out of 7 social and solidarity economy (SSE) facilities in the health zones of Buta (4) and Ganga (3) and briefing of 17 service providers and 1 hygiene worker (100%) on the wearing of PPE, waste management, sorting, hand washing and the preparation of chlorine solutions; no functional sorting in the structures visited, non-functional manual waste management (MWM) in Buta and no waste assessment carried out. in the context of a strengthened strike by service providers. In South Ubangi, service providers in the Bulu health zone were trained on IPC.

2.4.2. Challenges.

- **In Bas-Uélé,** only 130 out of 632 service providers (20.6%) are trained in IPC-WASH, 16 out of 20 priority health facilities lack functional triage, 17 out of 20 lack water for 72 hours, and no WASH partners support activities around cases. **In Haut-Uélé,** only 3 out of 7 health zones have submitted their data, IPC supervisors are not activated in Rungu, Dungu, Boma Mangbetu, and Gombari, and IPC kits are lacking for health facilities.

2.5. Holistic care

2.5.1. Main actions

In Ituri, 47 new admissions, all of suspected cases (including 8 admitted in advanced stages), were recorded, compared to 69 discharges, including 4 recoveries. Thus , 355 patients (130 confirmed and 225 suspected) are hospitalized, representing an occupancy rate of 34% (24% for confirmed beds and 44% for suspected beds); 333 out of 344 patients (96.8%) received hot meals, all children under LNPE/SLM were cared for (11/11 at the CTE, 4/4 in daycare and 51/51 in the community) and 20 patients were evacuated.

In North **Kivu,** 138 new admissions (13 confirmed and 125 suspected) were recorded, compared to 98 discharges, including 4 recoveries (Kalunguta, Oicha, Lubero, and Mutwanga), 92 non-cases, and 2 deaths of confirmed cases (Katwa and Vuhovi). A total of 541 patients (178 confirmed and 363 suspected, including 66 children under 5 and 6 critically ill) are receiving care, 179 of whom are outside of Ebola Treatment Centers (ETCs), for an ETC/CT occupancy rate of 85.4% (362/424). Five out of 16 ambulances (31.3%) completed 9 trips, with 8 successful completions (88.9%), and all 17 separated or orphaned non-breastfed infants are receiving standardized treatment (100%).

- **In Tshopo,** 1 new admission of suspected cases was recorded against 2 discharges of confirmed recovered cases, including one doctor; 14 patients (10 confirmed and 4 suspected) are hospitalized at the CTE Météo (occupancy rate: 38.9%; 14/36), including 2 in serious condition; 11 confirmed and 4 suspected cases received hot meals, as did the 51 PPLs being monitored, and the staff of the CT Kongakonga was reinforced with the support of the IMC.

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• **In Haut-Uélé,** no patient care data was reported for the day. In Bas-Uélé, 4 new suspected cases were admitted (2 isolated in the Bondo ESS and 2 in Buta) and 3 discharges of non-cases were recorded in Buta after negative results, bringing the number of isolated suspected cases to 51 due to the lack of a functioning CTE.

2.5.2. Challenges

In Ituri, the blood shortage persists, 32 patients are critical, 11 patients did not receive nutrition in Mandima, capacity remains low in Mambasa, Kilo, Lolwa, Mandima, and Biakato, and the Kasenyi Ebola Treatment Center (ETC) did not report due to a strike by healthcare providers (completeness: 28/29). In North Kivu, ETCs/CTs remain close to saturation (85.4%), 179 patients are being treated outside ETCs/CTs, 6 patients are critical, and the completeness of patient care data is only 79% (Butembo, Beni, and Musienene are missing); in Tshopo, 2 confirmed escapees from Bafwasende remain missing, a building at the Bafwasende ETC was burned down, and data entry is hampered by the multiplicity of data collection forms. In Haut-Uélé, no PEC data has been transmitted and the lack of nutritional inputs persists; in Bas-Uélé, the standardized CTE in Buta is still under construction and none of the 51 isolated suspects are receiving nutritional support.

2.6. Vaccination

2.6.1. Main actions

In North **Kivu,** 191 people living with HIV were vaccinated by two teams in the Ngilinga health district (Unichir secondary hospital, Beni health zone), bringing the total to 1,191 out of 7,105 expected (16.8%), with 1,409 doses remaining out of 2,600 received and no doses lost. **In Bas-Uélé,** 500 doses of Ervebo were received on October 31, 2026, and the deployment of a compassionate vaccination support team is being prepared in the Bili health zone; cumulatively, 1,972 out of 13,202 targeted individuals (14.9%) have been vaccinated in the province. **In Ituri,** no vaccinations were reported during the day. The cumulative total remains at 2,448 vaccinated individuals (1,036 women, including 26 pregnant and 72 breastfeeding women, and 1,412 men), with 35 non-severe AEFI and 5 severe AEFI reported since the beginning, and the TUMAINI, Partners, EBO-PEP and Oracle clinical trials are continuing.

2.6.2. Challenges

In Ituri, no PPL were vaccinated during the day, the TPL/PPL target remains to be redefined and 5 serious AFIs have been notified cumulatively; **in Bas-Uélé,** the start-up of the UCCs is still not effective, the support of field actors is delayed and the ZS of Bili still has no vaccinated (0/1067); **in North Kivu,** the coverage of PPLs in Beni remains low (16.8%), the 2 vaccination teams need 4 vehicles and 6 tents and 1 non-serious AFI has been notified since the start of vaccination.

2.7. Continuity of Care

2.7.1. Main actions • In North Kivu, the

baseline assessment of the Wanamahika CH and the Matanda secondary hospital revealed triage not respecting the circuit and without isolation, insufficient PPE, absence of emergency service, non-standard neonatology (only one incubator for 5 premature babies per month, without phototherapy), absence of RHZE and insufficient HIV testing, non-integrated nutritional care and only 25% of staff trained in IPC; the mapping of partners aligned with the CSSE was completed and 4 recovery plans for the assessed ESS were produced; In Tshopo, the visit to 3 ESS (CM Bel Ange, CM Emmanuel and PS Trinité) revealed the absence of trained providers, effective triage and up-to-date registers, with none of the 11 agents of PS Trinité being vaccinated, while 47 cases of malnutrition are being monitored by these ESS.

2.7.2. Challenges

Insufficient labile blood products and nutritional inputs at the Matanda secondary hospital; lack of a contingency plan for absenteeism and a dashboard to monitor service disruptions in the assessed health establishments in North Kivu, where emergency room visits have decreased since the start of the epidemic; non-standardized collection tools and ineffective triage in the health establishments visited in Tshopo; no data transmitted by Ituri, Haut-Uélé, Bas-Uélé and Sud-Ubangi.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

• **In Ituri,** 22,761 home visits reached 93,224 people, 282 educational talks reached 7,531 people, 46 mass awareness campaigns reached 2,837 people, 2 community debates reached 119 people and 11 advocacy campaigns reached 31 authorities and leaders; 367 community alerts, including 79 deaths, were notified, 6 out of 9 feedback (66.7%) and 7 out of 10 incidents (70.0%) were resolved within 48 hours; the CREC facilitated the pre-listing of 461 contacts around 16 confirmed cases, the decontamination of 48 out of 48 households, the completion of 19 out of 21 EDS (90.5%), the announcement of 16 out of 16 positive results and the transfer of 59 out of 73 suspects (80.8%).

In North Kivu, 16,237 home visits reached 56,136 people, 299 educational talks reached 6,213 people, 57 mass communication sessions reached 4,937 people, and 1 briefing reached 176 people; 441

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alerts out of 650 (67.8%) were notified, 29 feedbacks out of 37 (78.4%) and 8 incidents out of 24 (33.3%) were resolved within 48 hours and 2,404 people out of 4,750 (50.6%) were engaged in community actions; a public forum was organized in the Benengule AS (Beni) to address concerns and rumors, and 68 students from the IBTP in Butembo were sensitized.

- **In Haut-Uélé,** 268 home visits reached 1,876 people, 33 educational talks reached 363 people and 8 mass activities in the churches of Isiro, Pawa, Boma Mangbetu and Wamba reached 1,584 people; 6 community alerts, all resolved, and 3 death alerts were notified, 31 people linked to 3 suspected cases and 3 community deaths were sensitized to facilitate contact tracing, decontamination and EDS, 165 people were sensitized during EDS in Pawa and Wamba, and the CREC supervised the integration of Ebola messages into the distribution of mosquito nets (AS N'Sele, Gossamu and Mayogo) and the joint sensitization of passengers to PoCs.

- **In Tshopo,** 544 home visits reached 3,428 people, 7 educational talks reached 543 people and 43 mass awareness campaigns reached 126,697 people in markets, the 8 private beaches of Lubunga and Makiso and 32 churches, in addition to 3,054 students and teachers from 4 schools in Kisangani; 6 radio programs were broadcast, 163 posters and 206 advice cards were distributed, 3 out of 5 refusals were lifted in Bafwasende, 5 out of 5 feedbacks and 2 out of 2 incidents were resolved within 48 hours, the training of provincial CREC actors and ZS hotspots on infodemic management was closed and the discharge of two recovered patients was accompanied.

- **In Bas-Uélé,** 693 home visits reached 3,293 people, 4 educational talks reached 147 people and 11 mass awareness campaigns reached 3,134 people, 1 radio program was broadcast in Buta and CREC organised the exit ceremony and motorised caravan for the reintegration of a healed person in Dakwa as well as the community reintegration of two healed people in Ganga.

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- **In Ituri,** 15 affected health zones out of 28 (53.6%) were covered, 28 confirmed cases out of 32 (87.5%), 67 confirmed out of 130 and 117 suspected out of 188 under follow-up as well as 36 new suspected out of 43 were taken care of, the 7 households of separated and orphaned children (100%) were accompanied, 29 children followed, including 14 in nursery, 9 recovered out of 11 reintegrated and 21 out of 21 followed in community, and 64 results announced, including 28 positive; 257 accompanying persons and 73 PPL were supported.
- **In North Kivu,** 8 out of 9 health zones (88.9%) were covered, 21 out of 25 new cases (84.0%), 206 confirmed cases under follow-up, 38 new suspected cases and 245 suspected cases under follow-up were taken care of, 37 out of 40 (92.5%) separated and orphaned child households were supported, 6 out of 6 children were followed up in daycare and 28 out of 36 (77.8%) in the community, 12 recovered patients were reintegrated and 13 out of 13 are followed up in the community, 108 results were announced, including 28 positive results, 462 people (173 men, 227 women and 62 children) benefited from psychoeducation (UNICEF support) and 77 PPLs were supported; the training of members of the SMSPS pillar in the health zones of Kyondo, Vuhovi and Kalunguta was supported by WHO.

- **In Tshopo,** 6 out of 7 notified health zones (85.7%) were covered, the 10 confirmed cases being monitored at the CTE Météo received psychological support and the 4 suspected cases received psychoeducation, resistance to isolation was lifted at the Rekapi polyclinic (Kabondo), 21 people, including 8 caregivers and 2 PPLs, received psychoeducation on barrier gestures and the 2 recovered patients were reintegrated (2/2).

2.9.2. Challenges

- **In Ituri,** only 15 out of 28 affected health zones (53.6%) are covered, barely half of the confirmed cases under follow-up (67/130) are receiving support and coverage remains incomplete around the latest cases in Bunia (6/8).
  In North Kivu, SMSPS coverage remains weak around the latest cases in Biena, Manguredjipa, Kayna, Lubero, Masereka, Kalunguta, Mabalako, Mutwanga, and Vuhovi. No childcare facilities are available in 10 affected health zones, and the intervention team lacks logistical resources and reintegration kits for recovered patients. In Haut-Uélé, no SMSPS reports have been submitted, and no childcare facilities are available for separated children. In Tshopo, community health workers in the Wanie-Rukula health zone are not trained, and the intervention team lacks communication and transportation resources. In Bas-Uélé, psychosocial support for the three confirmed cases is still delayed.

2.10. Logistics

2.10.1. Main actions

In North **Kivu,** supplies destined for the Biakato Health Zone (Ituri), consisting of tents, tarpaulins, and portable toilets, were packaged on behalf of UNICEF, and service providers from the various pillars were deployed to the health zones; two health centers in Katwa received infection prevention and control (IPC) kits, 37 schools in Butembo were equipped, and 5,360 liters of water were provided in public places in Katwa and Oicha; 200 doses of health products and medicines were also distributed.

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doses from Ervebo have been received for the Beni health zone (cumulative: 2,600 doses received, 1,409 remaining, none lost) and the vaccination teams express a need for 4 vehicles and 6 tents.

• **In Tshopo,** for the PSM, the Kisangani laboratory remains in pre-break of MVE buffer, two modules of the Radione platform are broken and the faulty regulator of the solar system interrupts the power supply in the evening; logistics supported the mobility of the PCI, PEC, CREC and PoE/ PoC pillars for the deployment of inputs and the 3 available ambulances (100%) ensured 2 trips, all successful.

• **In Haut-Uélé,** 3 health establishments in Wamba (General Referral Hospital, Treatment Center and Matari Hospital) have been provided with IPC kits, but the kits remain insufficient for the other health establishments and 1 out of 5 Radi-One machines is out of order at the Isiro laboratory; in Bas-Uélé, 500 doses of Ervebo were received on October 31, 2026 and the construction of the Treatment Center at the Buta General Referral Hospital continues, but the 20 priority health establishments do not have mattresses and 17 out of 20 lack water storage tanks; in Sud-Ubangi, IPC inputs (PPE, chlorine), monitoring tools and communication materials remain insufficient and an urgent supply of essential medicines is requested.

2.11. Security

2.11.1. Main actions

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• **In North Kivu,** the PCI team was accompanied and secured for the decontamination of 5 households and the Benengule Health Center (Beni Health Zone); **in Ituri,** gunfire from unknown individuals prevented any health checks at the Iga Barrière Point of Control (Nizi); **in Tshopo,** response activities remain suspended in the Bafwasende Health Zone, where a CTE building was set on fire; **in Bas-Uélé,** the security situation related to the strike movement was assessed and activities were monitored at the zonal level.

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

2.12.1. Main actions

In North Kivu, 277 people were sensitized on the PSEA and reporting via 495555/151: 200 at the Letroc market in Beni, including 140 women, with the support of the WHO, 66 grassroots leaders and members of the CODESA of the AS of Mandje (Itebero), including 9 women, and 11 in Katwa; in Ituri, 24 repairers and carpenters from the Grâce divine and Kirukou workshops (18 men and 6 boys) were sensitized in Bunia and the display of messages and the distribution of PSEAH leaflets continue in the health zones of Bunia, Aru, Mongbwalu, Mambasa, Fataki and Mandima; PSEA data from the other provinces were not transmitted for the day.

2.12.2. Challenges

• Coverage of affected areas remains limited: only **North Kivu** and Ituri have reported their activities

PSEA for the day, only one partner (WHO) reports in Ituri and no leaflets were distributed during the awareness-raising in Mandje; the reporting mechanism is known only in 15 out of 36 health zones (41.7%) in Ituri, 8 out of 34 health zones (23.5%) in North Kivu and 7 out of 23 health zones (30.4%) in Tshopo; visibility materials remain insufficient for mass awareness-raising and CBCMs lack effectiveness.

KEY MESSAGES

No new health zones have been affected in the last 24 hours.

hours,

67 out of 178 health zones (37.6%) remain affected in eight provinces.

Sixty-five (65) new confirmed cases were reported in three

provinces, including 39 in North Kivu, 19 in Ituri and 7 in Haut-Uélé.

transmission remains particularly marked in the health zones of Beni (17 cases) and

from Mutwanga (11 cases) in North Kivu, from Bunia (5 cases) and from Mambasa (4 cases)

in Ituri and Pawa (3 cases) in Haut-Uélé.

Of the 23 confirmed deaths reported, 20 are community deaths

(11 in North Kivu, 8 in Ituri and 1 in Haut-Uélé) and 3 are deaths at CTEs

(2 in North Kivu and 1 in Ituri). The contact tracing rate is 84.0%.

(30,362/36,126), down from the previous day (86.6%) and below the threshold

of 95%, with the lowest rates in Tshopo (66.6%), where the strike of

Health personnel are disrupting follow-up care, and in North Kivu (79.0%). The situation

emphasizes the need to strengthen early detection and contact tracing

and dignified and safe burials.

TIMELINE OF KEY EVENTS

From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

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